Announcement
Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.

Dermatology 23S Module 1

Go to Week 5 Academic Forum

Day 2 task

Day 2 task
by Huma Jaffar (Tutor) - Sunday, 8 October 2023, 5:59 AM

Day 2 task
A 41-year-old man was referred to clinic with hyperhidrosis. He has no other history of note and takes no medication of note. He complains of sweating at the drop of a hat and at most times of the day, experiencing sweating on his forehead and also under the arms. It has become rather embarrassing for him. He has used potent antiperspirants but they offer little help. This has been a deteriorating problem for the last year or so. On examination, he is obese with a BMI of 34 kg/m2. There is little else of note. He was referred to clinic by his GP, who was wondering if there were any specific causes.

On the clinical portal he has had the following tests: FBC, LFTs, TFTs and HbA1c - normal

- Would you ask about anything else in the history?
- What would be the differential for hyperhidrosis?
- Are there any further investigations you would request?

156 words

Reply

Re: Day 2 task
by Pooran Outar - Monday, 9 October 2023, 4:26 AM

Case Discussion

My presumptive diagnosis: Primary Idiopathic Hyperhydrosis

Additional important detail to the current medical hstory:

1. When did the excessive sweating first appear (childhood or adolescence or recent), and has it been continuous (more than 6 months)
2. The areas of excessive sweating are noted but it does not mention if the sweating is bilateral and symmetric
3. If there are any triggering factors such as heat, physical activity etc.
4. Family history of excessive sweating
5. Any other co morbid conditions
6. Other unusual relevant symptoms or changes, such as weight loss, fever, or changes in skin appearance?

Differential diagnosis (Schwartz, 2019)**:**

1. Diabetes mellitus
2. Hyperthyroidism
3. Hyperpituitarism
4. Drug-induced (propranolol, physostigmine, pilocarpine, tricyclic antidepressants)
5. Chronic alcoholism
6. Hodgkin disease or tuberculosis
7. Frey Syndrome
8. Eccrine nevus
9. Eccrine angiomatous hamartoma

Here I have listed some differential diagnosis for Hyperhydrosis, which do you consider more important and what others do you think are pertinent?

Accoriding to Schwartz, 2019, **addtional aboratory tests** that could be included in this patient's work up may include:

1. Iodine and starch test to aid in diagnosis
2. Urinary catecholamines may reveal a possible pheochromocytoma.
3. Uric acid levels may reveal gout.
4. CXR to rule out tuberculosis or a neoplastic cause
5. Manteaux

Are these tests along with those mentioned in the case enough to arrive at a diagnosis or are there more specific tests to consider?

Reference:
Schwartz, R.A. (2019). Hyperhidrosis: Background, Pathophysiology, Etiology. *eMedicine*. [online] Available at: https://emedicine.medscape.com/article/1073359-overview.

231 words

Reply

Re: Day 2 task
by Rahima Abdulaziz Ahmed - Tuesday, 10 October 2023, 6:18 PM

Thank you Pooran for the discussion , the other tests that I may add include:
1.Cortisol level ; To assess for conditions like Cushing's syndrome
2.Tests for Diabetes , Although the HbA1c is normal, fasting blood glucose or an oral glucose tolerance test maybe considered to further evaluate glucose metabolism.
3.Additonal Hormonal testing ; depending on clinical suspicion , other hormonal tests may be relevant.

65 words

Reply

Re: Day 2 task
by Siddharth Saluja - Sunday, 15 October 2023, 10:10 PM

Hello both,
From the NICE guidelines for secondary hyperhidrosis if primary hyperhidrosis has been ruled out, we need to consider :
1) Full blood count.
2) Erythrocyte sedimentation rate and/or C-reactive protein.
3) Urea and electrolytes.
4) Liver function tests.
5) HbA1c.
6) Thyroid function tests.
7) Tests for HIV or tuberculosis, if indicated
8) Blood film for malarial parasites
9) 24-hour urine collection for catecholamines, metanephrines (to exclude phaeochromocytoma) and 5-hydroxyindoleacetic acid (to exclude carcinoid tumours).
10) Chest X-ray.
If a diagnosis of primary focal hyperhidrosis is suspected with no underlying cause identified, and specific diagnostic criteria are met, no routine investigations are needed in primary care.

References
NICE. (n.d.). CKS is only available in the UK. [online] Available at:
https://cks.nice.org.uk/topics/hyperhidrosis/diagnosis/assessment/.

122 words

Reply

Re: Day 2 task
by Adam Tollitt - Monday, 9 October 2023, 10:48 AM

Hyperhidrosis

Diagnosis

I suspect that this patient most likely has either hyperhidrosis secondary to obesity or has primary idiopathic hyperhidrosis that perhaps in the last year has been exacerbated by obesity.

The key features of the case that point towards one of these diagnoses include:

- Symmetrical distribution of hyperhidrosis (secondary causes/pathological commonly are asymmetrical or generalised)
- No history of concurrent medication use that might be contributory
- Otherwise asymptomatic

- Normal battery of biochemical investigations
- Elevated BMI 34 – potentially suggestive of obesity as a secondary cause

Important additional history

- Total duration/onset of symptoms. This is a key question in the history and would be important in differentiating between hyperhidrosis secondary to obesity and primary idiopathic hyperhidrosis exacerbated by obesity. Primary idiopathic hyperhidrosis nearly always starts in childhood/adolescence.
- Any specific triggers for hyperhydrosis. For example, if this patient's hyperhidrosis had a clear situational component (e.g. being in crowded places) this may point towards more of a secondary hyperhidrosis driven by underlying anxiety
- Exact distribution of sweating
- Presence of other symptoms that may suggest another secondary aetiology. In a younger patient it would be important to ask about possible symptoms of phaechromocytoma including headaches, visual disturbance, palpitations.
- Family history of hyperhidrosis. Primary idiopathic hyperhidrosis often runs in families or a positive family history for another condition may point towards a potential secondary cause.
- Use of over-the-counter medication/alcohol/caffeine intake which may be contributory
- Smoking history

(Dermnetnz.org, 2015; Dunford et al., 2022)

Differential diagnoses of hyperhidrosis

Hyperhidrosis can be categorised by whether it is primary (idiopathic) or secondary to another underlying condition. It can also be further categorised by its distribution (focal or generalised).

Primary = primary idiopathic hyperhidrosis

Secondary = focal vs generalised

Focal

- Stroke
- Neuropathy
- Peripheral nerve damage
- Spinal nerve damage
- Surgical sympathectomy - one of the potential radical treatments for hyperhidrosis that can cause compensatory focal hyperhidrosis in another body area
- Auriculotemporal syndrome (gustatory hyperhidrosis)

Generalised

- Obesity
- Type 2 diabetes mellitus
- Menopause
- Hyperthyroidism
- Respiratory failure

- Phaeochromocytoma
- Parkinsons disease
- Malignancy
- Hodgkins lymphoma
- Exogenous causes (alcohol, nicotine, caffeine, medications – SSRIs, Opioids, tricyclic antidepressants, corticosteroids)

(Dermnetnz.org, 2015; Dunford et al., 2022)

Additional investigations

As Pooran has outlined, there are numerous additional tests than can be undertaken in the work up for hyperhidrosis. In this specific case, providing that there were no other red flags highlighted in the additional history or any other symptoms suggestive of another secondary cause, I suspect additional investigations looking for these are not indicated. The key diagnostic information has been obtained from the initial history and biochemical test results.

It would be useful to consider checking the patient's blood pressure and additional bloods including lipid profile/U&Es. These would be useful in screening for evidence of cardiovascular disease and to calculate future potential cardiovascular disease risk.

If this patient was hypertensive and/or experiencing palpitations, I would also arrange an ECG and consider arranging referral to secondary care for investigation of phaeochromocytoma depending on the presence of other positive symptoms/degree of clinical suspicion.

Further discussion

Hyperhidrosis is a very common presenting complain in general practice and I find in many cases there is often a trigger/factor in the patient's history that may either fully explain or contributes significantly to their symptoms. My experience of managing patients with primary idiopathic hyperhidrosis has included reinforcing lifestyle/conservative management advice and use of topical aluminium chloride preparations. I have seen some patients who have been referred to secondary care started on anticholinergic medication including oxybutynin and glycopyrronium bromide, however many of these patients have discontinued their use due to intolerable side effects - commonly dry mouth. I have also encountered patients who have received botox injection treatment and iontophoresis treatment, but these have typically been undertaken in the private sector.

I would be interested to hear from the group about their experiences of seeing/managing patients with primary idiopathic hyperhydrosis and what treatment options people have experience of?

References

Dermnetnz.org. (2015). *Hyperhidrosis*. [online]. Available at: https://dermnetnz.org/topics/hyperhidrosis. (accessed 09/10/2023)

Dunford, L., Clifton, A.V., Stephenson, J., Radley, K., McDonald, L., Fretwell, L., Cheung, S.T., Hague, L. and Boyle, R.J. (2022). Interventions for hyperhidrosis. *Cochrane Database of Systematic Reviews*, 2022(2). doi:https://doi.org/10.1002/14651858.cd015135.

679 words

Reply

Re: Day 2 task
by Huma Jaffar (Tutor) - Monday, 9 October 2023, 9:58 PM

Excellent review of hyperhidrosis Pooran and Adam'

As you said it could be primary or secondary hyperhidrosis - do you expect primary hyperhidrosis in this 41 y/o, started about a year ago?
What are the criteria for primary hyperhidrosis?
Can someone tabulate and compare primary Vs secondary hyperhidrosis?

48 words

Reply

Re: Day 2 task
by Sophie Holloran - Tuesday, 10 October 2023, 11:12 AM

Thank you Adam and Pooran – I found your summaries really useful.

I think given that the history started 1 year ago and there are other factors that could cause secondary hyperhidrosis, I think this is much less likely to be primary hyperhidrosis.

NICE (2023) has the following diagnostic criteria for primary hyperhidrosis:

Diagnostic criteria for primary focal hyperhidrosis

Suspect a diagnosis of primary focal hyperhidrosis if a person has focal, visible, excessive sweating that:

- Occurs in at least one of the following sites: axillae, palms, soles of the feet, or craniofacial region, *and*
- Has lasted at least 6 months, *and*
- Has no apparent cause, *and*
- Has at least *two* of the following characteristics:
  - Is bilateral and relatively symmetrical.
  - Interferes with daily activities.
  - Episodes occur at least once per week.
  - Onset is before 25 years of age.
  - Positive family history.
  - Localized sweating stops during sleep.
- Note: if symptoms have lasted for less than 6 months or onset is at or after 25 years of age, primary focal hyperhidrosis is possible if other diagnostic criteria are met, but clinical judgement should be used to exclude an underlying cause.

Fig 1 - NICE CKS Hyperhydrosis

Does anyone else have any other diagnostic criteria?

66 words

Reply

Re: Day 2 task
by Pooran Outar - Tuesday, 10 October 2023, 2:30 PM

Hello Dr Huma,

My first impression was secondary hyperhyrldosis but given that there is no clear indication in the case when the symptoms first appeared I assumed that it could have been a late consultation and the patient may have been suffering from this disease for a number of years. This is of course guiding from my own experience as I have a friend who has primary hyperhydrosis but never sought medical attention until he was in medical school and learned about the disease. Furthermore, the absence of medical conditions other than obesity, medications and other external factors made me lean more towards primary hyperhydrosis. Also, it was noted that the areas affected in secondary hyperhydrosis cab affect various body parts but is often more generalized.

Nonetheless now that I understand that his worsening symptoms for about a year indicated the onset was also a year ago, I would agree secondary hyperhydrosis.

152 words

Reply

Re: Day 2 task
by Xiao Liey Ding - Tuesday, 10 October 2023, 3:37 PM

Hi Prof Huma,

Sophie has stating out the diagnostic criteria for primary hyperhidrosis.

Below are some useful clues to suggest primary vs secondary :

	Primary Hyperhidrosis	Secondary Hyperhidrosis
Distribution	Bilateral and relative symmetrical at following sites: axillae, palms, sole of the feet or craniofacial region	Generalize rather than focal
Age of Onset	Before 25	After 25
Family History	Positive family history	Usually none
Pattern	Sweating stops during sleep	Present nocturnally

Causes of secondary hyperhidrosis need to be considered in aggressive manner especially patient's symptoms started older than 25-year-old and has been a deteriorating problem for the past 1 year or so. Among the secondary causes are:

· Alcohol use (Check history from patient)

· Chronic pulmonary disease (Look for symptoms and signs)

· Congestive heart failure (Look for symptoms and signs)

· Endocrine/ metabolic disorder: diabetes, thyrotoxicosis, hypoglycaemia (Already ruled out by investigations)

· Infection such as tuberculosis (Need to check if patient has night sweat and other constitutional symptoms. Night sweat is strongly suggestive of secondary hyperhidrosis and is also a symptom of pulmonary tuberculosis)

· Malignancy: pheochromocytoma (Before setting expensive investigation of urine catecholamine, blood pressure and heart rate is useful bedside examination)

· Medications use such as SSRI, SNRI, cholinergic agonists (Patient is not taking any medication)

· Physiology changes such as in menopause (NOT applicable in this case as patient is a male)

· Psychiatric disease such as anxiety disorder (Mental state examination)

· Substance abuse or narcotic withdrawal (History and signs)

· Neurologic such as Parkinson disease or spinal cord injury (No history)

Reference:

John R. Mcconaghy and Daniel Fosselman. Hyperhidrosis: Management Options. Am Fam Physician. 2018;97(11):729-734

272 words

Reply

Re: Day 2 task
by Rahima Abdulaziz Ahmed - Tuesday, 10 October 2023, 6:02 PM

Tabulated comparison between primary and secondary hyperhidrosis

Characteristics	Primary Hyperhidrosis	Secondary hyperhidrosis
Underlying Cause	No underlying medical condition; often genetic or idiopathic	Caused by an underlying medical condition , medication or other factors
Age of onset	Typically begins in childhood or adolescence.	Can occur at any age , often appearing later in life.
Areas affected	Typically affects specific localized areas such as palms , soles , underarms or face.	Sweating can be generalized , affecting larger body areas.
Sweating Triggers	Sweating often occurs in response to emotional stress, heat or physical activity	Sweating maybe triggered by the underlying medical condition , medication or other factors
Family History	Often a positive family history of hyperhidrosis	May or mayn't have a family history of hyperhidrosis, depending on the underlying cause
Severity and Frequency	Can be severe and frequent interfering with daily life.	Severity varies depending on the underlying cause, may be intermittent
Common Underlying Causes	None; typically considered an isolated condition.	Various underlying causes, including medical conditions (e.g. infections, endocrine disorders, medications e.g antidepressants) and other factors e.g menopause.
Diagnostic Approach	Diagnosis based on clinical evaluation and exclusion of secondary causes	Requires identification and treatment of the underlying medical condition or trigger.
Treatment Options	Antiperspirants , topical treatments , Botox injections , and in severe cases surgery (sympathectomy) or iontophoresis	Treatment involves addressing the underlying cause or discontinuing medications. Symptomatic relief with antiperspirants or medications maybe considered
	Generally a lifelong condition but	Prognosis depends on the underlying condition;

Prognosis	Generally a lifelong condition but manageable with treatment	improvement or resolution may occur with successful treatment of the primary cause .

Its important to note that both primary and secondary hyperhidrosis can significantly impact an individuals quality of life , and appropriate evaluation and management are essential for symptom relief and improved well being .Hence in history its important still to ask about the social and psychological impact on patients quality of life, emotional well being and daily activities and manage accordingly and referral to a psychologist/psychiatrist where necessary.
Thank you all for your inputs.

References;

Smetana G W (2023) Uptodate Available at: https://www.uptodate.com/contents/evaluation-of-the-patient-with-night-sweats-or-generalized-hyperhidrosis

Oakley. A (1997) *Dermnet* Available at: https://dermnetnz.org/topics/hyperhidrosis Accessed; (10/10/23)

357 words

Reply

Re: Day 2 task
by Felica Mwinji Namukonde - Tuesday, 10 October 2023, 6:11 PM

hey, this is a well explained table about the difference between primary and secondary hyperhidrosis, i think in our practice so far i have only seen one patient with primary hyperhidrosis wr could not do much for the patient because of limited resources.

43 words

Reply

Re: Day 2 task
by Ayten Sadek - Monday, 16 October 2023, 2:22 AM

Thanks for this table differentiating between primary and secondary hyperhidrosis. It is so beneficial as it is clear and concise summary of the characteristics of both conditions. I have seen before a patient with primary hyperhidrosis and botox injections helped.

40 words

Reply

Re: Day 2 task
by Sadaf Basharat - Tuesday, 10 October 2023, 11:24 PM

Yes, I definitely would agree that it was an excellent review and at the same time i also strongly agree that primary hyperhidrosis is quiet unexpected at 41 years age.

Hyperhidrosis can be further divided into 2 types, depending mainly on their causes

Primary hyperhidrosis is a condition characterized by excessive sweating that typically affects specific areas of the body, such as the palms, soles, underarms, and face. It usually occurs without an underlying medical cause and often starts in childhood or adolescence.

Secondary hyperhidrosis, on the other hand, results from an underlying medical condition or medication side effects. It can cause generalized sweating throughout the body and may occur at any age. Treating secondary hyperhidrosis involves addressing the underlying cause or adjusting medications.

while primary hyperhidrosis may be managed with topical treatments, medications, iontophoresis or more invasive procedures like Botox injections or surgery.

144 words

Reply

Re: Day 2 task
by Jillian Simpson - Wednesday, 11 October 2023, 2:48 PM

I would want to explore the history some more.
I would want to know the exact location, is it only the forehead and axillae affected?
How long has this been going on for?
Has he noticed any triggers to it?
Is it worse when he sleeps?
Is it worse after certain foods or drinks?
I would also like to know if he is taking any medication or buying any drugs?
Does he drink alcohol?
Has he got any other past medical history?
Is there any family history?
Has he recently travelled anywhere? Does he have any other symptoms?

You could also assess his symptoms using the validated hyperhidrosis disease severity scale (HDSS) from NICE CKS (2023). This explores the severity of the sweating and how it interferes with his daily activities, a score of 1-2 points is regarded at mild-moderate and 3-4 points moderate-severe.

In terms of differentials, given his age if this was all new for him then there are other diseases I would want to explore. If the sweating was occurring more at night I would have tuberculosis or malignancy such as Hodgkin's lymphoma as other possible conditions to investigate. I note his TFT's and HBA1c are normal but if theses had not been requested I would have ordered them as sweating can be a symptom of hyperthyroidism and with his BMI diabetes also needs to be excluded, although secondary hyperhidrosis can be caused by obesity. We also need to consider Parkinson's disease and insulinoma as other differentials (PCDS, 2022).

Investigations I would order U&E's and CRP in addition to what has already been requested plus a chest X ray. I would want to check for infections such as HIV, TB. May consider blood film for malarial parasite depending on travel history. Also 24 hour urine collection for catecholamines, metanephines (to exclude phaeochromocytoma) and 5-hydroxyindoleacetic acid (to exclude carcinoid tumours).

In terms of clinical practice, we use oxybutynin modified release and glycopyrronium for the management of hyperhidrosis in our hospital after they have tried all first line treatments in primary care. We did use Botox at one point but in line with other health boards this is no longer offered locally.

NICE. (2023). Hyperhidrosis CKS. [online] Available at: https://cks.nice.org.uk/topics/hyperhidrosis/. [Accessed 11/10/23]
PCDS. (2022). Hyperhidrosis (excessive sweating). [online] Primary Care Dermatology Society. Available at: https://www.pcds.org.uk/clinical-guidance/hyperhidrosis. [Accessed 11/10/23]
Oakley, A. (2015). Hyperhidrosis | DermNet NZ. [online] Available at: https://dermnetnz.org/topics/hyperhidrosis. [Accessed 11/10/23]

401 words

Reply

Re: Day 2 task
by Felica Mwinji Namukonde - Tuesday, 10 October 2023, 5:30 PM

HYPERHIDROSIS HISTORY TAKING.

A detailed medical history is very important exclude underlying disease.

- Find out the characteristic of the sweating exact details are important in order to clarify severity and frequency including daytime symptoms.

- Fevers must be excluded with certainty if present record daily temperatures during and after sweating episodes.
- Risk factors for TB the patients should be evaluate including prior TB skin test, HIV infection, hemodialsis, gastroscopy or TB active symptom s like cough, sputum, fatigue and weight loss.
- Asses for constitutional symptoms b, lymphadenopathy, or a malignancy known history ask about weight loss, fatigue, and pruritis.
- HIV risk factors any injection drug use, unprotected sex, diarrhoea, thrush, unintentional weight loss
- Assess for bacterial infection risk for bacteremia, back pain associated with fever
- geographycal infection risk, travel history assess for malaria, typhoid, tick borne disease, food and water hygiene
- Hormonal; consider age and menopausal in women (oestrogen levels) in males any surgical histroy leading to andtrogen reduction., ask about endocrine symptoms flushing , wheezing, diarrhoea , headache palpitations
- Medication use sometimes medication can induce hyperhidrosis

Neurolgic history, any neurlogical injury associated with autonomic dysreflexia.

DDX for Hyperhidrosis:
Infectious include;
Tuberculosis
HIV infection
Bacterial infection (endocarditis, osteomyelitis, pyogenic abscess)
Brucellosis
Malaria
Malignancies:
Lymphoma
Solid non hematologic malignancies
Medications (antidepressants, cholinergic agents,hypoglycemics, estrogten)
Endocrine causes(menopause, endocrine disorders, hyperthyroidism, pheochromocytoma, carcinoid syndrome)
Neurologic disorders.

References;
Smetana G W (2023) *Uptodate* Available at: https://www.uptodate.com/contents/evaluation-of-the-patient-with-night-sweats-or-generalized-hyperhidrosis?search=hyperhidrosis&source=search_result&selectedTitle=2~111&usage_type=default&display_rank=2 Accessed : (10/10/23)

Oakley A (1997) *Dermnet* Available at: *https://dermnetnz.org/topics/hyperhidrosis* Accessed; (10/10/23)

244 words

Reply

Re: Day 2 task
by Oyeombiliyetu Jason - Tuesday, 10 October 2023, 6:23 PM

What to ask further on history?

In the above patient, I would like to find out if there are any changes in sexual behaviour (libido/ erection problems)- as low levels of testosterone in males can lead to general hyperhidrosis.

Explore B symptoms to rule out infections or malignancies.

Further characterization of the hyperhidrosis that the patient is experiencing (onset, duration, association to food intake, exertion, alcohol consumption and environmental changes, the exact areas affected, family history). Obesity is said to rarely cause general hyperhidrosis, however can worsen it.

Impact on the quality of life for the patient.

Suggests secondary hyperhidrosis

Short anamnesis

Symptoms of another disease which can give rise to secondary hyperhidrosis

Regional or asymmetrical sweating

Suggestion of primary hyperhidrosis

Long anamnesis

Early onset

Heredity

Focal, bilateral, symmetrical sweating

Sweating decreases or stops at night

Differentials

Physiological causes

Menopause

Pregnancy

Fever

Pathologic

Proliferative diseases: Myelodysplastic disorde, Hodgkin’s lymphoma

Endocrinopathies: Hyperthyroidim, Pheochromocytoma

Diabetes and hypoglycaemi, Carcinoid syndrome Hypopituitarism and pituitary tumour, Acromegaly

Cardiac diseases: Heart failur, Endocarditis

Infectious diseases: Influenz, HIV infectio, Tuberculosi, Encephalitis

Nervous system diseases: Pituitary strok, Post-traumatic spinal cord injurie, Parkinson’s diseas, Familial dysautonomi, Polyneuropathies

Metabolic disorders: Obesity

Psychiatric disorders: Alcoholis, Generalized panic disorde, Social phobia

Drug adverse reactions:

Opioid analgesics- Morphine, oxycodone, fentanyl, tramadol

Cyclooxygenase inhibitors Naproxen, nabumetone

Antibiotics and antivirals Ceftriaxone, ciprofloxacin, acyclovir

Cardiac and hypotensive medicaments Amlodipine, nifedipine, verapamil, carvedilol, metoprolol, enalapril, lisinopril, losartan, hydralazine, propafenone

Antidepressants and mood stabilizers Tricyclic antidepressants, neuroleptics

Hypoglycaemic agents Insulin, glipizide

Topically used formulas Glucocorticosteroids

(Kisielnicka A et Al. 2022)

Further investigations

Full blood count

Renal function test and electrolytes

Liver function test

Thyroid function test

Fasting glucose/ HbA1c

HIV/ TB/ Malaria test and other infective processes and infective markers

24 h-hour urinary catecholamines

Uric acid

Chest Xray

References

1. Rystedt A.et Al (2016) 'Hyperhidrosis- an unknown widespread "silent" disorder' Journal of Neurology and Neuromedicine 1(4):25-33

2. Benson RA. Et Al (2013) 'Diagnosis and management of hyperhidrosis' British Medical Journal 347(f6800): oh 28-31)

3. Kiseilnicka A. (2022) ' Hyperhidrosis: disease aetiology, classification and management in the light of modern treatment modalities. Advances in Dermatology and Allergology. 39(2):251-257. Available at: https://www.ncbi.nlm.nih.gov/pmc/articles/PMC9131949/. Accessed: 10 October 2023.

357 words

Reply

Re: Day 2 task
by [Arwaf Benrawwaf]() - Wednesday, 11 October 2023, 3:25 PM

Hyperhidrosis:

It’s an excessive sweating more than required for thermoregulation

In the history, it would be important to ask about any family history of hyperhidrosis or other medical conditions, as well as any recent changes in lifestyle or exposure to new environments or substances. We should ask the patient about other symptoms such as fever, wight loss, night sweats. (*eMedicine*2023).

There are deferent types of hyperhidrosis: (dermnetnz.org. 2015)

1.Primary focal hyperhidrosis:

Most common form and usually starts in childhood or adolescence. It is not associated with any underlying medical condition.

2. Secondary hyperhidrosis:

This is caused by an underlying medical condition or medication. Some possible causes include: Infections: such as tuberculosis or HIV.

Endocrine disorders: such as hyperthyroidism or diabetes.

Neurologic disorders: such as autonomic dysfunction or Parkinson's disease.

Medications: such as antidepressants or antipyretics.

Malignancies: such as lymphoma.

3. Generalized hyperhidrosis:

This involves excessive sweating throughout the body and can be caused by systemic conditions like menopause, obesity, anxiety disorders, or certain medications.

The differential diagnosis for hyperhidrosis includes: (*eMedicine*2023)

- Burning feet syndrome
- DM
- Gout
- Hodgkin disease
- Thyrotoxicosis
- POEMS syndrome
- Dermatologic manifestations of Glomus tumor

Further investigations that may be requested include: (*eMedicine*2023)

1.Skin biopsy:

This may be done to rule out any skin conditions that could be causing the hyperhidrosis.

2. Sweat test:

(*eMedicine*2023)

This test measures the amount of sweat produced by the body and can help differentiate primary focal hyperhidrosis from secondary causes.

3. Labs:

If there is suspicion of an endocrine disorder, hormone levels may be checked e,g, cortisol level to roll out Cushing's syndrome.

Blood glucose level and Thyroid function test should be part of screening

Uric acid if gout is suspected

4. Imaging studies:

Depending on the suspected underlying cause, imaging studies such as chest X-ray or CT scan may be requested.

5. Neurologic evaluation:

If there are symptoms suggestive of a neurologic disorder, a neurologist may be consulted for further evaluation.

Reference:

Hyperhidrosis: Background, Pathophysiology, Etiology. (2023). *eMedicine*. [online] Available at: https://emedicine.medscape.com/article/1073359-overview?form=fpf&scode=msp&st=fpf&socialSite=google&icd=login_success_gg_match_fpf [Accessed 11 Oct. 2023].

dermnetnz.org. (n.d.). *Hyperhidrosis | DermNet NZ*. [online] Available at: https://dermnetnz.org/topics/hyperhidrosis.

350 words

Reply

Re: Day 2 task
by Huma Jaffar (Tutor) - Thursday, 12 October 2023, 9:48 PM

Good Take up everyone !
Do consider Use of illicit drugs – cocaine – very common and often not asked!
You may also ask Erectile dysfunction, reduced shaving frequency, change in ring size maybe and Testosterone – in this case obesity-related hypogonadism with low testosterone!

42 words

Reply